جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : دينا سيد محمد ابوزيد
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** ش محمد الخولي
رقم البطاقة : 29112150102865 **رقم التليفون :** 01096968017 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 15-12-1991 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: دينا سيد محمد ابوزيد
National ID: 29112150102865

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



٠١٠٩٧٩٦٨٠١٧



جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة

08/23/2026

MDT FORM

Name:	[illegible]	MRN:	11581
Age:	35 Y	ID:	29112150102805
Menopausal Sta		Phone:	1096980019
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: Invasive duct carcinoma

ER: 8/8 PR: 6/8 HER2: 1 KI67: 20

Sonomammography:
sono mamo right breast multiple suspicious mass lesions as described with UOQ suspicious microcalcifications ipsilateral pathological LEVEL1& II axillary lymph nodes.

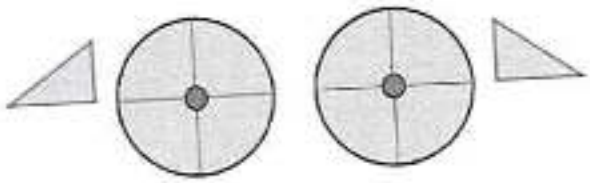
Mets Workup:
MRI spine showed dorsal spine lesion
bone scan requested
MRI to be handeled to MDT

History:

Recommendations:

MDT Outcome & Plan
For femara /zoladex
palliative rth to D7 lesion

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat					

Name: دنيا سيد محمد ابو زيد **MR. :** 11581	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented with bilateral lower-limb weakness and urinary incontinence. An MRI film was obtained via phone but no formal report was available. Cord compression was suspected, and Dr. Shymaa Lashin was consulted for urgent referral to a neurological service for emergency evaluation.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : امنية محمد محمد عيسوي

يعتمد ,

Patient Name	**دينا سيد محمد 0**	**Code Number**	**YK 252000**
Sex/Age	**Female 34 Y**	**Patient ID**	**18893**
Registered Date	**18/5/2026 12:13:53 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**08/6/2026 08:09:08 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**10/6/2026 12:47:28 PM**	**Pathology No.**	**1559/26**

Clinical Data: Right breast UOQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 18/05/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9) & SISH.

Microscopic examination:
-**Histologic type:** Invasive duct carcinoma.
- **Histologic grade: 2 (**differentiation 2, pleomorphism 2, mitosis 2)
- **DCIS:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** Mild.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 15% of tumor cells (score 6/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- SISH: Not amplified. (Average Her2 copy No.: (1.95), Her2/CEP17 ratio: (1.2)).
- Ki67%: 20%.

Conclusion:
Right breast mass, Tru-cut biopsy:
-Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 6/8).

Patient Name	**0 دينا سيد محمد**	**Code Number**	**YK 252000**
Sex/Age	**Female 34 Y**	**Patient ID**	**18893**
Registered Date	**18/5/2026 12:13:53 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**08/6/2026 08:09:08 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**10/6/2026 12:47:28 PM**	**Pathology No.**	**1559/26**

- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
(Average Her2 copy No.: (1.95), Her2/CEP17 ratio: (1.2)).
- Ki67%: 20%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
***Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahim Ghoneim

Patient ID: 95421	Patient Name English: Dina Said Mohamed
Age: ٣٤ سنة, ٤ شهر	2026/05/14 12:28:28 PM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 34 years old female patient with right breast lump 1 month duration , negative family hx of breast cancer .

Detailed Findings

Right breast:

- Hetergenously dense fibro glandular breast parenchyma . (ACR density C).
- The right breast shows UOQ posterior 1/3 ill-defined spiculated dense mass lesion with obscured margins and architectural distortion seen in both CC & Mlo Views about 10.5 cm from the nipple.
- Ultrasound revealed large ill defined hypoechoic mass lesion measures about 11 x 8.8 mm at its maximum diameters located at 10 ° clock position . Associated with punctate foci of calcifications .vabnormal vascularity detected .
- Normal nipple/areolar complex.
- **Multiple enlarged axillary lymph nodes at level I with focal uniform cortical thickening measuring 6 mm and others show effaced fatty hila largest one measures about 22 x 12.8 mm at level I.**
- No Evidence of suparaclavicular or infraclavicular lymphnodes .

Left breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Multiple small dispersed subcentimetric thin-walled anechoic cysts.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 4): Right UOQ suspicious mass lesion , suspciousd microcalcifications associated with ipsilateral pathological axillary LEVEL I as described. For Tru cut biopsy and dynamic MRI breast study.**
- **Left breast (BIRADS 2): left breast fibrocystic changes.**

Much obliged,
Dr. Sarah Hussein MD

Patient ID: 55421	Patient Name English: Dina Said Mohamed
Age: ٣٢ سنة, ٢ شهر	2026/05/14 12:28:28 PM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 34 years old female patient with right breast lump 1 month duration , negative family hx of breast cancer .

Detailed Findings

Right breast:

- Heterogeneously dense fibro glandular breast parenchyma.(ACR density C).
- The right breast shows UOQ posterior 1/3 multiple spiculated dense mass lesions with overlying suspicious microcalcifications about 10.5 cm from the nipple. Another oblong lesion is seen in the lower inner quadrant .
- Complementary Ultrasound revealed speculated hypoechoic mass lesion measuring about 1.5 x 1.4 cm at its maximum diameters located at 10 ° clock position zone c with another similar yet smaller lesion seen posterior measuring 8 x 4.4 to it with distance 2.6 cm, the lesion is seen resting on the muscle with no clear plane of cleavage for further MRI assessment. Another lesion is seen anterior to the index lesion with distance 1.3 cm measuring 13.5 x 11.6 mm
- Similar lesions are seen 8-9 o'clock measuring 10.8 x 7.6 mm & 11.3 x 8 mm and at 1 o'clock zone a measuring 8.7 x 8.6 mm and at 4 o'clock zone a measuring 11 x 6.5 mm
- A rather well defined oblong hypoechoic lesion is seen at 12 o'clock measuring 9.5 x 5 mm .
- Normal nipple/areolar complex.
- **Multiple enlarged axillary lymph nodes at level I some show focal cortical thickening measuring 6 mm and others show effaced fatty hila with non uniform outline suggesting extracapsular extension, the largest one measures about 22 x 12.8 mm at level I.**
- **Another single globular node with loss of hilum is seen at level II measuring 13.7 x 9.2 mm**
- No Evidence of supari-clavicular lymph nodes .

Left breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.

- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Multiple small dispersed subcentimetric thin-walled anechoic cysts.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 5): Right breast multiple suspicious mass lesions as described with UOQ suspicious microcalcifications ipsilateral pathological LEVEL I & II axillary lymph nodes. For Tru cut biopsy and dynamic MRI breast study.**
- **Left breast (BIRADS 2): left breast cystic changes.**

Much obliged,
Dr. Heba Azzam, MD
Dr. Sarah Hussein MD

Patient's Name:	**Dina Said Mohammed Abu Zeid**
Date:	**2026-08-11**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

NECK UTRASONOGRAPHY EVALUATION REVEALED: -

Thyroid: -

- *Both thyroid lobes and isthmus are of average size, smooth surface contour and homogenous echopattern*
- *Color Doppler interrogation revealed average vascularity of both lobes.*
- *Right thyroid nodule, hypoechoic, solid measuring 5x6 mm.*
- *Left thyroid cystic nodule measuring 5x6 mm.*

Lymph nodes: -

- *Left auricular and posterior auricular LNs showing benign sosnographic criteria largest measures 2x1.5 cm. oval in shape and faint hilum.*

Vascular structures: -

- *Intact vascular structures of the neck with no gross abnormality noted.*

Opinion: -

- *Right thyroid nodule TIRADS 4 for follow up.*
- *Left thyroid colloid nodule.*
- *Left cervical lymphadenopathy for follow up.*

Dalia Adel

FIRST SCAN
RADIOLOGY CENTER

فرست سكان
للأشعة والرنين المغناطيسى

Prof. Mohamed El Toukhy
Prof. Tamer A. Kamal

Prof. Moharram El Badawi
Prof. Khaled El Shantaly

Prof. Magdy Bassiouni

Patient ID: 94089	Ref. Phy.: القصر العيني
Report Date: 18/08/2026 05:53:34 AM	Patient Name: ديانا سيد محمد
Acc No: 162168	Age: 34 Years

MRI Study of the Dorsal Spine

Technique

- *Sagittal T1 and T2 weighted images.*
- *Axial T1 and T2 weighted images.*
- ***Post contrast series.***

Findings



- Infiltrative expansile marrow lesion eliciting low T1, iso T2 and high STIR signal intensity with homogenous post contrast enhancement, is seen involving D7 vertebral body with mild retropulsion indenting the ventral subarachnoid space & also involving part of its Left transverse process with small adjacent Left para-vertebral soft tissue component measure 21x8 mm.
- Similar marrow infiltrative lesion with similar signal intensity pattern but with marginal enhancement is also seen involving part of D8 vertebral body.
- D5 vertebral body osseous lesion of fat signal intensity with no enhancement suggestive of Hemangioma
- Normal alignment of the dorsal spine.
- Normal dimensions of the bony spinal canal.
- Normal size and signal pattern of the spinal cord.
- No evidence of posterior disc lesions.

CONCLUSION:

Marrow infiltrative lesions as described likely neoplastic.

Dr. Abdel Rahman Mohamed, MSc *Prof. Dr. Moharram El Badawi*

٩٢ - ٧ شارع متحف المنيل (امتداد كوبرى معهد الأورام) - القاهرة ت : ٥٠٠ / ٤٠٠ / ٣٠٠ / ٢٥٣٢٦٢٠٠
E-mail: firstscancenter.cairo@gmail.com ٠١٢٢٦٦٩٤٤٤٣ / ٠١٠ ٥٠٠٣ ٣٧٨٦ / ٠١٢٢ ٥١٠ ٣٣٣٣
0122 601 8000

Patient's Name:	**Dina Said Mohammed Abu Zeid**
Date:	**2026-06-24**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Right breast heterogenous parenchyma with multiple right axillary lymph nodes largest measuring 24 x 18 mm for sono-mammography.
- Prominent left axillary lymph nodes.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Two bilateral adnexal cysts measuring 39 mm on the right side and 24 mm on the left side for sonographic assessment.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D8 vertebral body lytic area is seen.

Opinion:

❖ **No pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy

Patient's Name:	**Dina Said Mohammed Abu Zeid**
Date:	**2026-08-11**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MRI EXAMINATION OF THE BRAIN

Technique of examination:

- **Axial T1, T2 & Flair WIs "Plate 1-3".**
- **Sagittal T1 WIs "Plate 4".**
- **No contrast was given.**

MRI Findings:

- Normal size shape & position of the cerebral ventricles with no midline shift or deformity.
- Normal gray–white matter interface with no areas of abnormal signal intensity.
- No evidence of mass lesions.
- No gross vascular abnormality.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- Normal extra axial spaces.

Opinion:

- **Normal MRI study of the brain.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

Patient's Name:	**Dina Said Mohammed Abu Zeid**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MRI Findings:

- Straightened lumbar curve.
- Normal sagittal diameter of the bony lumbar spinal canal.
- Diminished height and bright T2 signal intensity of the L4-5 lumbar disc denoting degeneration.
- L4-5 diffuse posterior disc bulge is seen effacing the epidural fat, flattening the theca and encroaching on the lower aspect of the corresponding exit neural foramina.
- L4-5 modic changes type I.
- L1 vertebral body lesion is seen of low SI at T1WI and intermediate SI at T2WI with post contrast enhancement most likely metastasis.
- L2 vertebral body tiny hemangioma.
- Normal marrow signal pattern of the lumbar vertebrae.
- Normal MRI appearance of the conus medullaris and cauda equine nerve roots.
- No pre-vertebral or para-vertebral soft tissue signal abnormalities.

Opinion:

- **L4-5 diffuse posterior disc bulge.**
- **L1 vertebral body lesion most likely metastasis.**
- **L2 vertebral body tiny hemangioma.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

M.R.I
[[DOCTYPE:MRI]]

Patient's Name:	**Dina Said Mohammed Abu Zeid**
Date:	**2026-08-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MRI EXAMINATION OF CERVICAL SPINE

-

Technique of examination:

- **Sagittal T1 & T2 WIs "Plate 1 & 2".**
- **Axial GR/T2 WIs "Plate 3".**
- **IV contrast was given.**

MRI Findings:

- Straightened cervical lordosis mounting to reverse.
- Degenerated cervical discs in the form of decreased discs height with lost bright T2WI SI.
- Spondylodegenerative changes of the cervical spine with anterior marginal osteophytes.
- C3-4 bilateral posterolateral disc protrusion compressing both neural foramina.
- C4-5 down to C6-7 diffuse posterior disc bulge effacing the ventral epidural space compressing both neural exit pathways and touching the cord at C6-7 with altered cord signal.
- Normal diameters of the bony cervical spinal canal.
- Normal MRI appearance of the cranio-cervical junction with no evidence of cord malacia, edema or syrinx formation.
- Normal marrow signal of the imaged vertebrae.
- No paraspinal signal abnormalities.

Opinion:

- **Spondylodegenerative changes of cervical spine with multilevel discs lesions as detailed.**
- **No suspicious bony lesions.**

Much Obliged;
Dr. Mai Shaaban MD

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

Technique of examination:

- **Sagittal T1 & T2 WIs "Plate 1 & 2".**
- **Axial T2 WIs "Plate 3".**
- **IV contrast was given.**

M.R.I
[[DOCTYPE:MRI]]

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs. / Mr .....دنيا سيد محمد..., 35.. years old

female/ male patient presented with diagnosis .....

.....MBC to bone

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She / He has received (3D/ IMRT) radiotherapy to.....

.....D6 - Dg.....

.....

At Dose : 20 .Gy/..5..fx/...1...w .

Additional (Boost) ; yes/ no. at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: 6. /9./2026

Ended: 10 /9./2026 With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.go.hi.com ت : 23913366 -23911838- ف :23914455 3411شارع برج سعيد -- باب الخلق -- القاهرة

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]